

CLINICAL INTELLIGENCE TREND REPORT

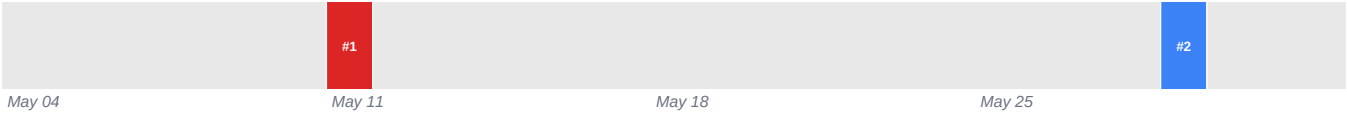
GREEN: Routine Review

Patient ID: ParkerPeriod: May 4 to June 1, 2026Report Date: June 2, 2026Coverage: 655/658h (100%)

Patient's monitoring period was less than 30 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 29 days from May 04 to Jun 01

HRBreathing

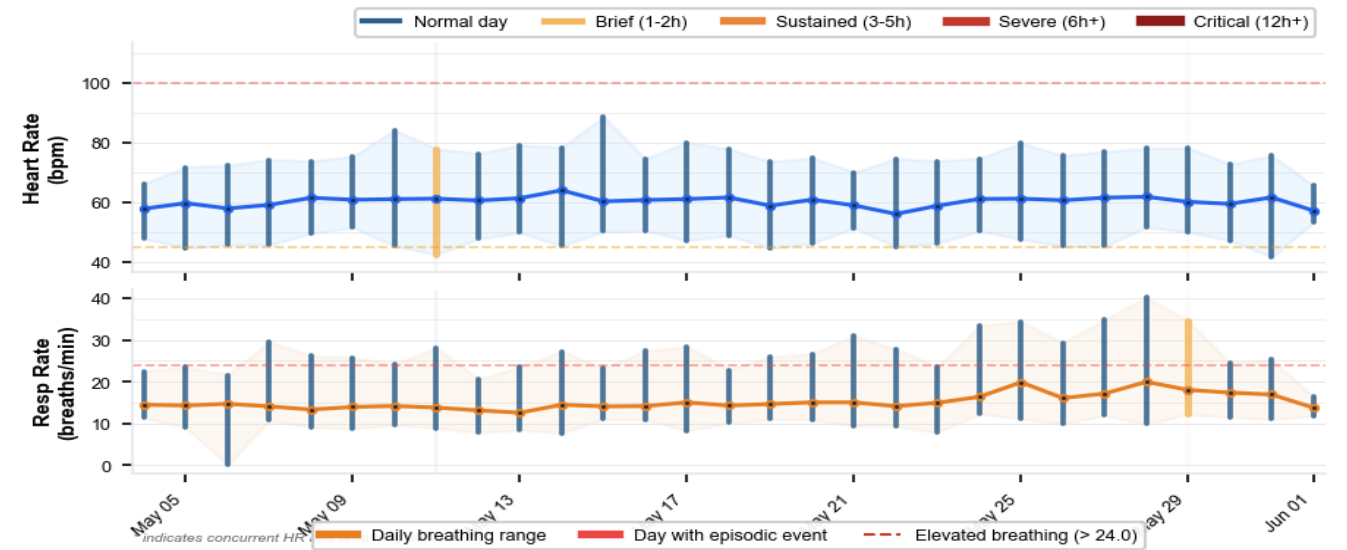


Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
2	2h	Low Heart Rate	<1	100%

Major Findings: Stable baseline

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	Low Heart Rate	42 bpm	1h	1h	1	45 bpm	May 11
2	Elevated Breathing	35 brpm	1h	1h	1	25 brpm	May 29

Daily recorded hours declined from 9.0 to 1.0 (89%) over the monitoring period.



Red bars indicate days with detected episodic events.
■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

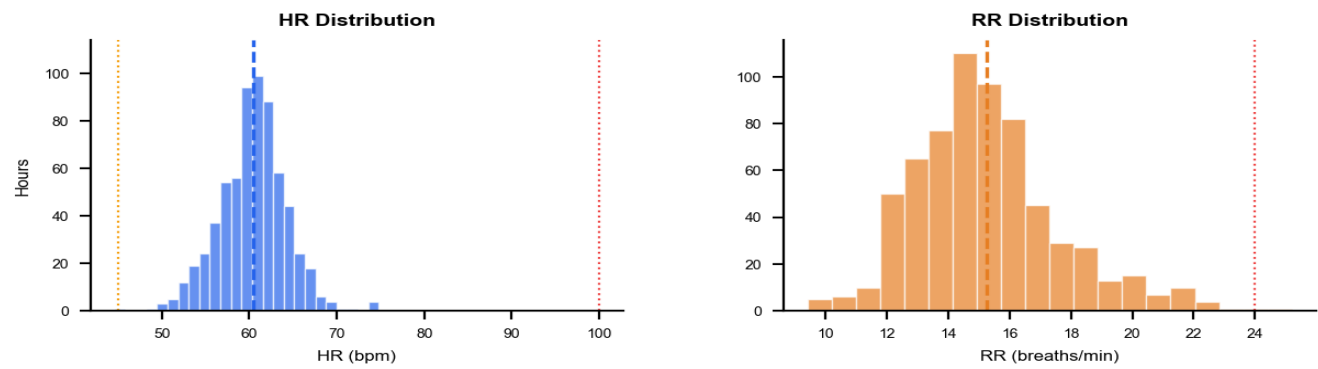
High Breathing > 30 brpm

High HR > 100 bpm

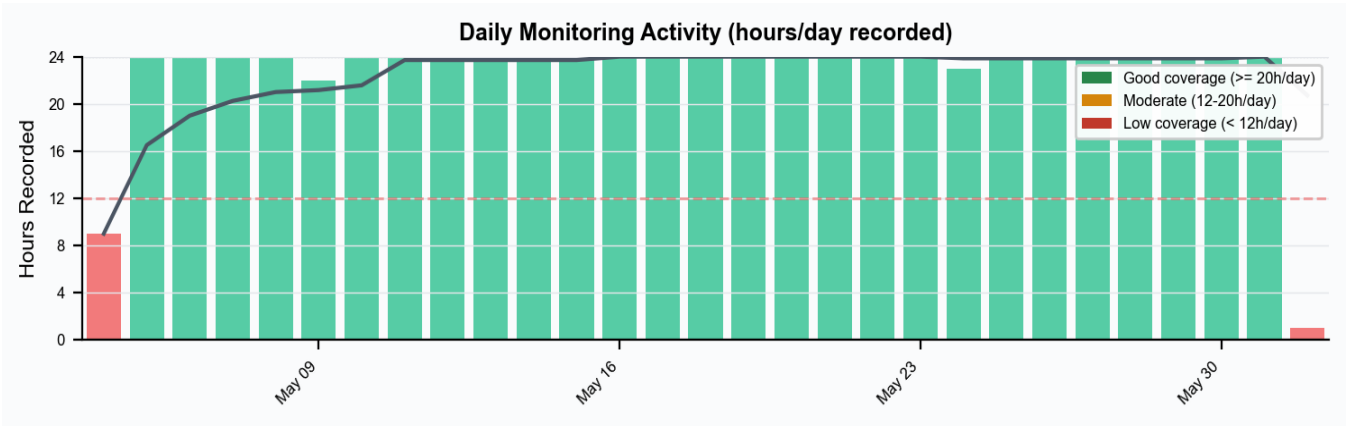
Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.